NCCP 00102 · VERSION 6

Your treatment, explained

Advanced melanoma · NCCP 00102 v6 · Document revised 6 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Vemurafenib alone — advanced melanoma

Reference treatment cycle

Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact the oncology team promptly for fainting or palpitations, eye pain or blurred vision, a blistering or rapidly spreading rash, severe sunburn, persistent vomiting or diarrhoea, reduced urine or yellow skin or eyes. Report new or changing skin growths even when the melanoma is responding.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp · zelboraf-pi

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
Your treatment phases
Your 28-day timeline
Baseline and cycle-specific checks
Whole treatment, scans and checks
Tablets, visits and practical planning
Understanding your diagnosis and treatment goal
Your medicines and overall plan
Taking or receiving treatment safely
Tests, results and treatment decisions
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Skin, body changes and daily care
Fertility, pregnancy and breastfeeding
Decisions, support and useful records
After a treatment hold, change or completion
Supportive medicines
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Questions to ask
Sources

UNDERSTAND YOUR TREATMENT

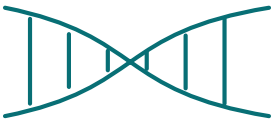
What each medicine does

To control advanced BRAF V600-mutated melanoma with a daily oral targeted medicine; benefit and tolerability are reviewed for the individual.

BRAF TARGETED TREATMENT

Vemurafenib

Zelboraf



Blocks an altered BRAF signal

Vemurafenib blocks the altered BRAF V600 protein that can drive melanoma-cell growth. A confirmed tumour BRAF V600 result is needed. Healthy tissues can also be affected; tablets need the same careful monitoring as treatment given in hospital.

Blocks an altered BRAF signal → Treatment plan and safety checks → Report new symptoms early

What to watch for

Report new skin growths, sunburn-like reactions, eye pain or blurred vision, palpitations or fainting.

Sources: nccp · zelboraf-pi

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Head / fatigue

Vemurafenib

Headache and tiredness can occur. New severe headache, confusion, collapse or neurological changes need urgent assessment rather than attribution to tablets.

3 Heart rhythm

Vemurafenib

The electrical rhythm can change; ECG and blood chemistry checks help detect risk. Palpitations or fainting need prompt assessment.

5 Liver

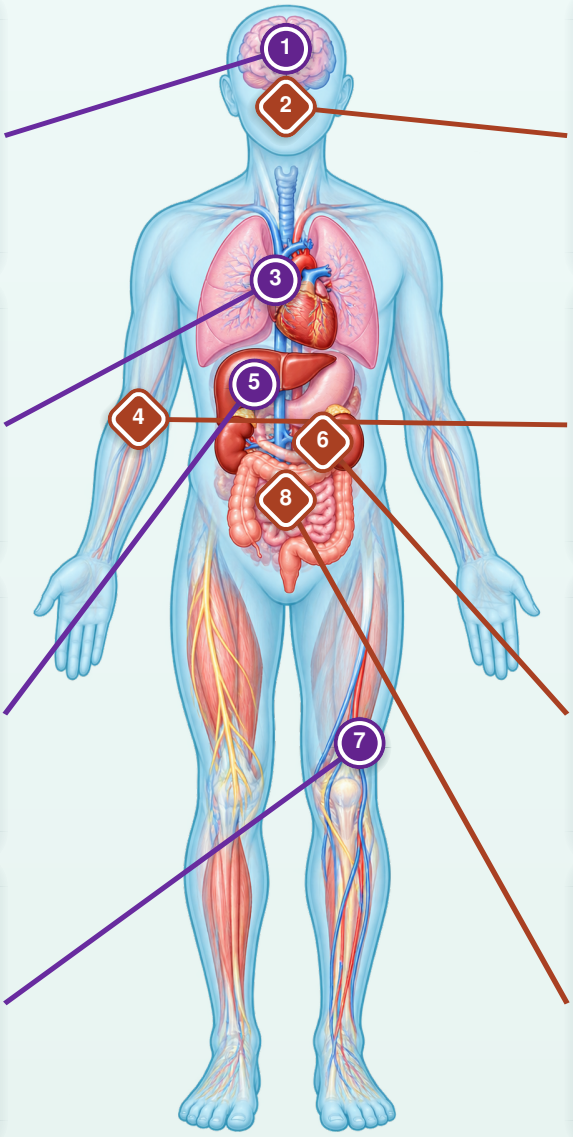
Vemurafenib

Blood tests check for liver injury. Yellow skin or eyes, dark urine or severe upper-abdominal discomfort need prompt review.

7 Joints / muscles

Vemurafenib

Joint or muscle pain can affect daily activities. Tell the team if pain limits walking or self-care, or occurs with marked weakness.



2 Eyes

Vemurafenib

Eye pain, redness, light sensitivity or blurred vision can signal inflammation. Report promptly for eye assessment.

4 Skin / sun

Vemurafenib

Photosensitivity, rash and new skin cancers can occur. Protect skin and report new growths; blistering or peeling is urgent.

6 Kidneys / fluids

Vemurafenib

Kidney injury or dehydration can affect dosing. Report reduced urine or inability to keep fluids down.

8 Stomach / bowel

Vemurafenib

Nausea, diarrhoea and reduced appetite can cause dehydration. Keep the agreed symptom plan and report persistent or severe symptoms.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

Your treatment phases

28-DAY REFERENCE CYCLE

Ongoing oral treatment

Ongoing oral treatment **Repeat only as prescribed**

Vemurafenib twice daily, about 12 hours apart, every day. A 28-day review cycle does not mean a planned tablet-free week.

Vemurafenib twice daily, about 12 hours apart, every day. A 28-day review cycle does not mean a planned tablet-free week.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00102 v6: current regimen source · EMA Zelboraf product information

THE WHOLE TREATMENT JOURNEY

Vemurafenib alone — advanced melanoma: treatment and checks

Advanced melanoma

Vemurafenib twice daily, about 12 hours apart, every day. A 28-day review cycle does not mean a planned tablet-free week.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00102	Confirm BRAF V600, blood count, kidney/ liver tests, medicine interactions and skin assessment. Current Zelboraf product information calls for a baseline ECG and electrolytes including magnesium for all patients; this supplements the NCCP risk-based ECG row. Staging scans establish the sites of melanoma.	Blood count, kidney and liver tests before each cycle. NCCP specifies ECG every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Its skin-review row states week eight then every 12 weeks; current Zelboraf product information instead requires monthly prescriber skin examinations. The team should supply a reconciled written skin-check plan using the current product information.	Contact the oncology team promptly for fainting or palpitations, eye pain or blurred vision, a blistering or rapidly spreading rash, severe sunburn, persistent vomiting or diarrhoea, reduced urine or yellow skin or eyes. Report new or changing skin growths even when the melanoma is responding.	Current Zelboraf product information calls for monthly skin checks during treatment and for up to six months after stopping, or until another anticancer therapy starts. Head-and-neck checks every three months and chest CT every six months during treatment screen for other cancers; these are distinct from the individual melanoma-response scan plan. Anal/pelvic checks are considered at baseline/ end and when indicated. Record the team's coordinated surveillance plan rather than duplicating scans.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00102	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts NCCP 00102	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1

Ongoing oral treatment

28-day reference cycle

To control advanced BRAF V600-mutated melanoma with a daily oral targeted medicine; benefit and tolerability are reviewed for the individual.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00102

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.42

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Vemurafenib twice daily, about 12 hours apart, every day. A 28-day review cycle does not mean a planned tablet-free week.

Treatment timing: displayed days 1–28

Treatment or check	Days in this window	What happens
Daily tablets / capsules	Days 1–28	Vemurafenib twice daily, about 12 hours apart, every day. A 28-day review cycle does not mean a planned tablet-free week.
Safety and symptom review	Days 1–28	Blood count, kidney and liver tests before each cycle. NCCP specifies ECG every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Its skin-review row states week eight then every 12 weeks; current Zelboraf product information instead requires monthly prescriber skin examinations. The team should supply a reconciled written skin-check plan using the current product information.

Schedule and review: Vemurafenib twice daily, about 12 hours apart, every day. A 28-day review cycle does not mean a planned tablet-free week.

Holds, missed treatment and phase changes: Take a missed dose only if at least four hours remain before the next dose. Otherwise skip it. Do not double a dose. If you vomit after taking it, do not take an extra dose; take the next dose at the usual time. Contact the team if vomiting continues or treatment has been held.

> **Why these bands stay broad:** This is a reference schedule. Your prescribed dose, blood results, symptoms and the team's written plan determine whether treatment goes ahead. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00102 v6: current regimen source ↗ · EMA Zelboraf product information ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Complete timing and symptom notes

Day 1–28: At home each day

The usual starting dose is 960 mg twice daily, about 12 hours apart; the prescribed dose may differ after review. Swallow tablets whole with water. Vemurafenib can be taken with or without food, but do not regularly take both daily doses on an empty stomach. Keep a consistent routine and follow the pharmacy label. Take a missed dose only if at least four hours remain before the next dose. Otherwise skip it. Do not double a dose. If you vomit after taking it, do not take an extra dose; take the next dose at the usual time. Contact the team if vomiting continues or treatment has been held.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

The usual starting dose is 960 mg twice daily, about 12 hours apart; the prescribed dose may differ after review. Swallow tablets whole with water. Vemurafenib can be taken with or without food, but do not regularly take both daily doses on an empty stomach. Keep a consistent routine and follow the pharmacy label.

STEP 2

Missed, vomited or held treatment

Take a missed dose only if at least four hours remain before the next dose. Otherwise skip it. Do not double a dose. If you vomit after taking it, do not take an extra dose; take the next dose at the usual time. Contact the team if vomiting continues or treatment has been held.

STEP 3

Appointments and supply

Arrange blood tests, transport, medicine supply and interpreter or carer support. Check who reviews results and how you receive the decision before the next dose.

STEP 4

Paper copies and contacts

Keep the full handbook, three-page summary and wallet card together. The QR code is optional. Ask the team to replace every labelled example contact with your real oncology and out-of-hours details; ask for a new printed plan when treatment changes.

Sources: NCCP 00102 v6: current regimen source. Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Supportive medicines and symptom plan

This is a minimal-emetic-risk oral regimen in the NCCP plan. Anti-sickness medicine is chosen to match symptoms and interactions; do not add over-the-counter medicines without checking. Agree a hydration and diarrhoea plan, and report inability to drink or persistent symptoms.

2 Skin and daily care

Use broad-spectrum UVA/UVB sun protection, protective clothing and lip protection; the product information specifies SPF 30 or higher. Sun sensitivity can be marked. Check skin regularly for new growths or changes, and keep scheduled dermatology reviews.

3 Fertility, pregnancy and breastfeeding

Use effective contraception during treatment and for at least six months after the final dose. Vemurafenib may reduce the effectiveness of hormonal contraception, so ask for an appropriate alternative or additional method. Discuss fertility and breastfeeding before treatment; tell the team immediately if pregnancy is possible.

Sources: nccp · zelboraf-pi

TREATMENT REFERENCE · 01

Understanding your diagnosis and treatment goal

Use your own pathology, stage and agreed plan alongside this guide.

Your diagnosis and stage

Keep the pathology report, original melanoma site, stage and known sites of spread. This guide concerns advanced disease that cannot be removed completely by surgery or has spread; it does not describe a postoperative recurrence-prevention course.

Sources: nccn-patient

The tumour test

A BRAF V600 test helps identify whether BRAF-targeted medicines are an option. A tumour mutation does not by itself mean an inherited family condition. Keep the exact result and ask how it changes your choices.

Sources: nccn-patient

Scans and local treatments

Record the scans used to follow known disease. Surgery or radiotherapy may have a separate role for a particular site; ask for their timing and risks to be added to the overall plan.

Sources: nccn-patient

New changes still matter

Report a new lump, changing mole, persistent skin sore or unexplained symptom. A response at one site does not establish that every other symptom is harmless.

Sources: nccn-patient

TREATMENT REFERENCE · 02

Your medicines and overall plan

To control advanced BRAF V600-mutated melanoma with a daily oral targeted medicine; benefit and tolerability are reviewed for the individual.

The schedule

Vemurafenib twice daily, about 12 hours apart, every day. A 28-day review cycle does not mean a planned tablet-free week.

Sources: nccp · zelboraf-pi

How treatment works

Vemurafenib blocks the altered BRAF V600 protein that can drive melanoma-cell growth. A confirmed tumour BRAF V600 result is needed. Healthy tissues can also be affected; tablets need the same careful monitoring as treatment given in hospital.

Sources: nccp · zelboraf-pi

Important boundaries

This is vemurafenib monotherapy. It is not the cobimetinib combination and has no cobimetinib seven-day break. The team must plan any radiotherapy because BRAF treatment can increase radiation-related reactions.

Sources: nccp · zelboraf-pi

Evidence and choices

At the 2011 analysis, six-month overall survival was 84% versus 64%; response was 48% versus 5%. These are group results from that study, not a prediction for you. Early historical analysis; later crossover and subsequent treatment affect longer-term comparisons. Dacarbazine comparator, not a modern BRAF/MEK combination or checkpoint regimen.

Sources: brim3 · nccp

TREATMENT REFERENCE · 03

Taking or receiving treatment safely

Keep the written prescription and actual treatment record together.

Administration

The usual starting dose is 960 mg twice daily, about 12 hours apart; the prescribed dose may differ after review. Swallow tablets whole with water. Vemurafenib can be taken with or without food, but do not regularly take both daily doses on an empty stomach. Keep a consistent routine and follow the pharmacy label.

Sources: nccp · zelboraf-pi

Missed or held treatment

Take a missed dose only if at least four hours remain before the next dose. Otherwise skip it. Do not double a dose. If you vomit after taking it, do not take an extra dose; take the next dose at the usual time. Contact the team if vomiting continues or treatment has been held.

Sources: nccp · zelboraf-pi

Other medicines

Give the team a complete list of prescribed, over-the-counter and herbal medicines, supplements and allergies. Ask before adding a new medicine; treatment interactions and symptom-masking can matter.

Sources: nccp · zelboraf-pi

Practical arrangements

Know where and when bloods are taken, how results reach the prescriber, and how to obtain the next supply or appointment. Ask for written instructions in a format you can use without a phone.

Sources: nccn-patient

TREATMENT REFERENCE · 04

Tests, results and treatment decisions

Safety tests and scans answer different questions.

Before treatment

Confirm BRAF V600, blood count, kidney/liver tests, medicine interactions and skin assessment. Current Zelboraf product information calls for a baseline ECG and electrolytes including magnesium for all patients; this supplements the NCCP risk-based ECG row. Staging scans establish the sites of melanoma.

Sources: nccp · zelboraf-pi

During treatment

Blood count, kidney and liver tests before each cycle. NCCP specifies ECG every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Its skin-review row states week eight then every 12 weeks; current Zelboraf product information instead requires monthly prescriber skin examinations. The team should supply a reconciled written skin-check plan using the current product information.

Sources: nccp · zelboraf-pi

What a result means

Ask which result is abnormal, whether the finding needs a repeat test and whether treatment is held, changed or stopped. Do not use a reference range or one normal result to decide independently that a dose is safe.

Sources: nccp · zelboraf-pi

Response and follow-up

Current Zelboraf product information calls for monthly skin checks during treatment and for up to six months after stopping, or until another anticancer therapy starts. Head-and-neck checks every three months and chest CT every six months during treatment screen for other cancers; these are distinct from the individual melanoma-response scan plan. Anal/pelvic checks are considered at baseline/end and when indicated. Record the team's coordinated surveillance plan rather than duplicating scans.

Sources: nccp · zelboraf-pi

TREATMENT REFERENCE · 05

What to expect between visits

A quiet treatment day does not rule out later problems.

Act on new symptoms

Contact the oncology team promptly for fainting or palpitations, eye pain or blurred vision, a blistering or rapidly spreading rash, severe sunburn, persistent vomiting or diarrhoea, reduced urine or yellow skin or eyes. Report new or changing skin growths even when the melanoma is responding.

Sources: nccp · zelboraf-pi

Supportive medicines

This is a minimal-emetic-risk oral regimen in the NCCP plan. Anti-sickness medicine is chosen to match symptoms and interactions; do not add over-the-counter medicines without checking. Agree a hydration and diarrhoea plan, and report inability to drink or persistent symptoms.

Sources: nccp · zelboraf-pi

A useful daily record

Record symptoms, temperature when unwell, bowel changes, fluid intake when reduced, missed doses and any medicines used for relief. Note how symptoms affect eating, walking, sleep and normal activities; this helps the team judge severity.

Sources: nccp · zelboraf-pi

When life is affected

Tell the team about fatigue, pain, appetite, sleep, mood and ability to work or care for others. Practical and supportive care can accompany anticancer treatment; you do not have to wait until treatment ends.

Sources: nccn-patient

TREATMENT REFERENCE · 06

Skin, body changes and daily care

Use the detailed body map to locate symptoms, not to diagnose them.

Skin and sun

Use broad-spectrum UVA/UVB sun protection, protective clothing and lip protection; the product information specifies SPF 30 or higher. Sun sensitivity can be marked. Check skin regularly for new growths or changes, and keep scheduled dermatology reviews.

Sources: nccp · zelboraf-pi

The body map

The organ anchors show possible locations and warning symptoms. They do not show how common each effect is, and a symptom may have another cause. New symptoms should be assessed in their clinical context.

Sources: nccp · zelboraf-pi

Food, fluids and activity

Keep meals and fluids manageable and follow any individual fluid or diet restrictions. Gentle activity and rest can be adjusted to how you feel. Ask for dietetic or rehabilitation help if intake, weight, strength or mobility is falling.

Sources: nccn-patient

Keep checking the plan

A new medicine, fever, hospital admission, procedure or radiotherapy may change treatment timing. Tell the oncology team and take the wallet card or medicine list to other appointments.

Sources: nccp · zelboraf-pi

TREATMENT REFERENCE · 07

Fertility, pregnancy and breastfeeding

Discuss these issues before the first dose whenever possible.

Contraception and pregnancy

Use effective contraception during treatment and for at least six months after the final dose. Vemurafenib may reduce the effectiveness of hormonal contraception, so ask for an appropriate alternative or additional method. Discuss fertility and breastfeeding before treatment; tell the team immediately if pregnancy is possible.

Sources: nccp · zelboraf-pi

Fertility preservation

Ask whether treatment or the cancer may affect fertility and whether fertility preservation is appropriate before treatment. A change in periods or sexual function does not reliably establish infertility.

Sources: nccp · zelboraf-pi

Written dates and methods

Record the agreed method, last treatment date and how long precautions continue. Different medicines have different intervals, so ask for clarification after a switch or combination change.

Sources: nccp · zelboraf-pi

Sexual health

Tell the team about sexual symptoms, relationship concerns or practical difficulties with the agreed precautions. Ask for a private discussion and specialist support when needed.

Sources: nccn-patient

TREATMENT REFERENCE · 08

Decisions, support and useful records

Your priorities belong in the treatment discussion.

Benefits and alternatives

Ask what the treatment aims to achieve, what alternatives are suitable, how likely important benefits are in your situation and how treatment burden or uncertainty affects that choice. A trial average cannot promise an individual outcome.

Sources: brim3 · nccp

Support alongside treatment

Ask about symptom control, psychological support, finances, transport, work and help at home. Palliative or supportive care can be used alongside anticancer treatment to improve comfort and function.

Sources: nccn-patient

A person you trust

If helpful, bring a family member or supporter and agree what information may be shared. Ask for an interpreter or accessible information when needed; use teach-back to check the schedule and urgent plan.

Sources: nccn-patient

Your document set

Keep consent information, the three-page summary, complete handbook, treatment card, test plan and real contact details together. The QR opens more information but essential instructions remain available in print.

Sources: nccn-patient

TREATMENT REFERENCE · 09

After a treatment hold, change or completion

The next step should be written down, not assumed.

Follow-up and late effects

Current Zelboraf product information calls for monthly skin checks during treatment and for up to six months after stopping, or until another anticancer therapy starts. Head-and-neck checks every three months and chest CT every six months during treatment screen for other cancers; these are distinct from the individual melanoma-response scan plan. Anal/pelvic checks are considered at baseline/end and when indicated. Record the team's coordinated surveillance plan rather than duplicating scans.

Sources: nccp · zelboraf-pi

Restarting or changing treatment

Take a missed dose only if at least four hours remain before the next dose. Otherwise skip it. Do not double a dose. If you vomit after taking it, do not take an extra dose; take the next dose at the usual time. Contact the team if vomiting continues or treatment has been held.

Sources: nccp · zelboraf-pi

A treatment summary

Record medicines and doses received, important side effects, admissions, last-dose date, ongoing symptom treatment and the clinician responsible for follow-up. Ask for an updated paper copy for your GP and other treating teams.

Sources: nccn-patient

Your next review

Confirm the next blood test, scan and review, who explains the result, and which symptoms should trigger contact sooner. Keep actual daytime and out-of-hours numbers accessible after treatment stops.

Sources: nccn-patient

What the treatment evidence can and cannot tell you

Evidence needs to match the medicine, dose combination, disease stage and previous treatment.

At the 2011 analysis, six-month overall survival was 84% versus 64%; response was 48% versus 5%. These are group results from that study, not a prediction for you.

Sources: brim3 · nccp

PREPARE FOR THE CONVERSATION

Questions worth asking

What is the treatment schedule?

Vemurafenib twice daily, about 12 hours apart, every day. A 28-day review cycle does not mean a planned tablet-free week.

What if a dose is missed or held?

Take a missed dose only if at least four hours remain before the next dose. Otherwise skip it. Do not double a dose. If you vomit after taking it, do not take an extra dose; take the next dose at the usual time. Contact the team if vomiting continues or treatment has been held.

Which symptoms need early contact?

Contact the oncology team promptly for fainting or palpitations, eye pain or blurred vision, a blistering or rapidly spreading rash, severe sunburn, persistent vomiting or diarrhoea, reduced urine or yellow skin or eyes. Report new or changing skin growths even when the melanoma is responding.

Which tests are needed?

Confirm BRAF V600, blood count, kidney/liver tests, medicine interactions and skin assessment. Current Zelboraf product information calls for a baseline ECG and electrolytes including magnesium for all patients; this supplements the NCCP risk-based ECG row. Staging scans establish the sites of melanoma. Blood count, kidney and liver tests before each cycle. NCCP specifies ECG every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Its skin-review row states week eight then every 12 weeks; current Zelboraf product information instead requires monthly prescriber skin examinations. The team should supply a reconciled written skin-check plan using the current product information. Current Zelboraf product information calls for monthly skin checks during treatment and for up to six months after stopping, or until another anticancer therapy starts. Head-and-neck checks every three months and chest CT every six months during treatment screen for other cancers; these are distinct from the individual melanoma-response scan plan. Anal/pelvic checks are considered at baseline/end and when indicated. Record the team’s coordinated surveillance plan rather than duplicating scans.

Can I use this without a phone?

Yes. The complete handbook, three-page summary and cut-out wallet card are printable. Ask the team to enter actual contact numbers and provide an updated paper plan when anything changes.

Try explaining the plan back: Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00102 v6. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00102 v6: current regimen source
- EMA Zelboraf product information
- NCCN Guidelines for Patients: Melanoma, 2026 — supplied education reference
- Chapman et al. BRIM-3, NEJM 2011

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-06.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6723/102_v6_Vemurafenib_Monotherapy.pdf

https://www.ema.europa.eu/en/documents/product-information/zelboraf-epar-product-information_en.pdf

https://www.nccn.org/patients/guidelines/content/PDF/melanoma-patient.pdf

https://www.nejm.org/doi/full/10.1056/NEJMoa1103782